

Clinical Preparedness Report

Counterfactual Risk Assessment with Evidence Attribution

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OVERALL RISK ASSESSMENT

HIGH

The patient's risk is rated as high due to her chronic stable mood disorder secondary to a medical condition, active suicidal ideation, aggression, and imminent homelessness, all of which significantly impact her mental health and risk of harm to self or others. [OVERRIDE: Risk tier elevated to HIGH due to critical safety signals detected in clinical note: CRITICAL SIGNALS DETECTED: suicide (1 signal), violence (3 signals), self_harm (1 signal). Risk override: HIGH.]

24

Evidence Spans

5

Nexus Factors

18.9%

Note Coverage

2/3

Active Scenarios

CRITICAL SAFETY ALERTS

Suicide/Self-Harm Risk:

- SUICIDE/'jump out' — ...nt with medications or following up with outpatient treators. She threatened to jump out her 10 story window if the took her to one of her outpatient treators. Her boyf...
- SUICIDE/'self-injury' — ...s and treatment: none Medication and ECT trials: Risperdal for Chorea movements Self-injury: denies Harm to others: denies Access to weapons: denies Trauma: Endorses s...

Violence/Aggression Risk:

- 'aggression' — ...] year old woman with [REDACTED] Disease who presented to the ED for worsening aggression, irritability, impulsivity, refusal to take medication and difficulty caring...
- 'aggression' — ...as pleasant and cooperative and did not show any irritability, impulsivity or aggression. She is being discharged to an assisted living facility that specializes in pa...
- 'aggression' — ...d some prn Risperdal in the event that she has experiences future episodes of aggression/instability. We also reduced her risk of harm to self/others, by d/c'ing Ms....

TOP RISK FACTORS

- [MODERATE] Chronic stable progression of mood disorder

2. [HIGH] Moderate suicidal ideation with active threats
3. [LOW] Aggression and refusal to take medication
4. [HIGH] Imminent homelessness and housing loss
5. [LOW] Significant weight loss and functional decline

PROTECTIVE FACTORS

- ✓ Engagement with a multidisciplinary team including psychiatrists and social workers
- ✓ Medication with Risperdal for Chorea movements
- ✓ Reduced risk of harm to self/others by discontinuing Ms. [REDACTED]'s medication

Evidence Color Legend



Branch A — Psychiatric



Branch B — Substance Use



Branch C — Social



Nexus — Multi-Branch

Evidence Attribution Table

Detailed breakdown of clinical evidence supporting each risk scenario branch.

ID	Risk Factor	Evidence from Clinical Note	Branch	Severity
1	functional_decline; warning_sign	"e: PSYCHIATRY Allergies: No Known Allergies / Adverse Drug Reactions Att..."	A, C	MODERATE
2	warning_sign	"with PMH of [REDACTED] Chorea and no known psych history,"	C, A	MODERATE
3	weight_change	"friend) and with a normal appetite. When asked what she normally ate she repl..."	A	LOW
4	Social Isolation Risk: No	"t know why because she has been eating normally. She did look very thin o"	C	LOW
5	housing_crisis; causal_step_1	"ned her head away and would not answer any questions. She is trying to go t..."	C	HIGH
6	critical_signal_suicide	"jump out"	A	HIGH
7	trauma_history; Trauma History: traum...	"te has more details from Uncle. Past Medical History: PAST PSYCHIATRIC HIS..."	A, C	MODERATE
8	clinician_prognosis	"ver a year [REDACTED] disease Social H"	A	LOW
9	functional_decline	"arly constant, subt"	A	LOW
10	functional_decline	"speech latency"	A	LOW
11	warning_sign	"ongue [REDACTED] Motor: MAEE with antigravity strengt"	A	LOW
12	causal_mechanism_1	"CTED] Movement exam: frequent choreiform movements in her trunk, shoulders an..."	C	LOW
13	warning_sign; functional_decline	"Tricycl-NEG [REDACTED] 11:50AM URINE Color-St"	A	LOW
14	critical_signal_violence	"aggression"	A	LOW
15	Trauma History: trauma/PTSD	"as pleasant and cooperative and did not show any irritability, impulsivity ..."	C	MODERATE
16	Housing Crisis: homelessness, imminen...	"SOCIAL The team coordinated care w/ the pt's family who were supp"	C	HIGH
17	clinician_prognosis	"difficulty w/ verbalizing thoughts/feelin"	A	LOW
18	clinician_prognosis	"TED], she was seen to be interacting we"	A	LOW
19	Significant weight loss, thin/emaciated	"e has been calm and in very good behavioral control throughout the entirety ..."	C	LOW
20	clinician_prognosis	"She does, however, have a high risk of"	A	LOW

ID	Risk Factor	Evidence from Clinical Note	Branch	Severity
21	Chronic stable Mood Disorder secondar...	"of harm to self/others, by d/c'ing Ms. [REDACTED] to a facility where she wi..."	A	LOW
22	warning_sign	"te. 1. RISperidone 0.75 mg PO"	A	LOW
23	warning_sign; Significant weight loss...	"et Refills:*0 2. RISperidone 0.25-0.5 mg PO BID:PRN agitation RX *risperidon..."	A, C	MODERATE
24	warning_sign; Social Isolation Risk: No	"CTED] Disease Discharge Condition: Mental S"	C, A	MODERATE

Clinical Note with Evidence Highlights

■ Branch A: Psychiatric

■ Branch B: Substance

■ Branch C: Social

■ Nexus: Multi-Branch

Name: [REDACTED] Unit No: [REDACTED]

Admission Date: [REDACTED] Discharge Date: [REDACTED]

Date of Birth: [REDACTED] Sex: F

Service: **PSYCHIATRY**^[A,C]

Allergies:^[A,C]

No Known Allergies / Adverse Drug Reactions^[A,C]

Attending: [REDACTED]^[A,C]

Chief Complaint:^[A,C]

Increased agitation, impulsivity, and difficulty caring for self^[A,C]

Major Surgical or Invasive^[A,C] Procedure:

None

History of Present Illness:

Patient is a [REDACTED] yo Female **with PMH of [REDACTED] Chorea and no**^[C,A]
known psych history^[C,A] presents to the ED on request of family who
are worried that she is depressed and has increased agitation.

On examination today, patient was sitting calmly in bed and was
aware that psychiatry was consulted. When asked how she has been
doing lately she only replied "fine." Her cooperation with the
interview was limited and progressed into refusal to answer
questions. She said she has been sleeping normal [REDACTED] hours per
boy^[A] **friend) and with a normal appetite. When asked what she**^[A]
normally ate she replied "bagels with peanutbutter and jelly,^[A]
[REDACTED] muffins, tuna, and egg salad." She did acknowledge she^[A]
has^[A]
lost weight(30lbs^[A] per ER phsycian) but doesn't **know why because**^[C]
she has been eating normally. She did look very thin o^[C]n exam.

Her family says she has not left the house in over a year. When
asked about why, she replied "I don't like the cold." When asked
why she didn't leave the house during the summer, [REDACTED] or [REDACTED]
times, she did not have an answer. During her days, the only
activity she would admit to was watching tv and playing with her
dog. When trying to probe into what else she does, she only
replied "I don't know." She denied any anxiety or phobia
symptoms. She said "I don't know" when asked about how her mood
has been for the last few weeks.

It seemed as though her chorea movements got worse throughout

the

interview as she got more irritated with me asking questions.

Towards the end of my interview, she turned her head away and would not answer any questions.

She is trying to go to a [REDACTED] facility in [REDACTED] for [REDACTED] but she is required to have a 3 midnight admission in a hospital beforehand. Her apt has been auctioned off and will be homeless at the end of the week.

Collateral from Uncle:

Mentioned patient was physically assaulting boyfriend at night, worsening emotional/behavioral outburst the last few weeks. She has not been compliant with medications or following up with outpatient treaters. She threatened to jump out her 10 story window if the took her to one of her outpatient treaters. Her boyfriend of [REDACTED] years has decided to leave her because he is not longer able to handle her. Dr. [REDACTED] note has more details from Uncle.

Past Medical History:

PAST PSYCHIATRIC HISTORY:

Hospitalizations: Denies

Current treaters and treatment: none

Medication and ECT trials: Risperdal for Chorea movements

Self-injury: denies

Harm to others: denies

Access to weapons: denies

Trauma: Endorses sexually molested in childhood, saw a psychiatrist

PAST MEDICAL HISTORY:

[REDACTED], MD but [REDACTED] seen in over a year [REDACTED] disease

Social History:

[REDACTED]

Family History:

No FHx of psychiatric disorders. Her mother was diagnosed as having [REDACTED] disease at the age of [REDACTED]. She died at the age of [REDACTED]. Her brothers and sisters do not have [REDACTED] disease. The patient does not have any children.

Physical Exam:

PHYSICAL EXAM:

- General Appearance: very thin middle aged woman sitting in

chair with intermittent, but nearly constant, subtle choreiform movements.

- NEUROLOGIC:

[REDACTED] Mental Status: awake, alert and oriented. Speech is extremely

dysarthric with prolonged speech latency^[A], but does improve somewhat when discussing topics that appeal to her. Often speaks using monosyllabic phrases.

[REDACTED] Cranial Nerves: grossly intact. Frequent frowning, grimacing facial movements and frequent episodes of tonic upward gaze.

Motor impersistence of the tongue^[A]

[REDACTED] Motor: MAEE with antigravity strength^[A], not assessed in detail

[REDACTED] Movement exam: frequent choreiform movements in her trunk, shoulders and more distally^[C] in her hands and feet. Frowning and grimacing facial movements as described above.

[REDACTED] Gait: wide-based and ataxic with enhanced choreiform hand movements in association. However, overall her gait base seems fairly stable since she also ambulates rather slowly.

- PSYCHIATRIC:

[REDACTED] Behavior: constant choreiform movements, not very cooperative with interview

[REDACTED] Mood: "fine"

[REDACTED] Affect: constricted, limited ability to assess given constant

dystonic/choreiform facial movements, but does seem to show some appropriate emotions (smiling/frowning etc).

[REDACTED] Thought process: linear, goal oriented, no LOA

[REDACTED] Thought content: denies SI, HI, AVH

[REDACTED] Insight/Judgment: good/poor

Pertinent Results:

ADMISSION LABS:

[REDACTED] 02:45PM BLOOD WBC-6.3 RBC-3.96* Hgb-13.3 Hct-41.1

MCV-104* MCH-33.6* MCHC-32.4 RDW-12.6 Plt [REDACTED] 02:45PM BLOOD Neuts-72.2* [REDACTED] Monos-4.6 Eos-1.8

Baso-0.5

[REDACTED] 02:45PM BLOOD Glucose-110* UreaN-14 Creat-0.7 Na-139

K-3.8 Cl-105 HCO3-27 AnGap-11

[REDACTED] 02:45PM BLOOD ALT-14 AST-13 AlkPhos-45 TotBili-0.2

[REDACTED] 02:45PM BLOOD VitB12-426 Folate-5.4

[REDACTED] 02:45PM BLOOD TSH-1.1

[REDACTED] 02:45PM BLOOD ASA-NEG Ethanol-NEG Acetmnp-NEG

Bnzodzp-NEG Barbitr-NEG Tricycl-NEG^[A]

[REDACTED] 11:50AM URINE Color-S^[A] raw Appear-Clear Sp [REDACTED] 11:50AM URINE Blood-MOD Nitrite-NEG Protein-NEG

Glucose-NEG Ketone-NEG Bilirub-NEG Urobilin-NEG pH-6.5 Leuks-MOD

[REDACTED] 11:50AM URINE RBC-2 WBC-3 Bacteri-FEW Yeast-NONE Epi-1

[REDACTED] 11:50AM URINE bnzodzp-NEG barbitr-NEG opiates-NEG

cocaine-NEG amphetm-NEG mthdone-NEG

Brief Hospital Course:

[REDACTED] is a [REDACTED] year old woman with [REDACTED] Disease

who presented to the ED for worsening **aggression^[A]**, irritability,

impulsivity, refusal to take medication and difficulty caring

for herself at home secondary to progression of her

[REDACTED].

PSYCHIATRIC

#) [REDACTED] Disorder with secondary Mood Dysregulation

Ms. [REDACTED] was admitted to the [REDACTED] inpatient psychiatry

unit for stabilization and re-initiation of home antipsychotics

for her mood symptoms. In the ED she was observed to be somewhat

agitated and uncooperative. However after restarting her

Risperidone, her hostility resolved and she was compliant and

cooperative throughout her stay. She was noted to have

near-constant choreiform movements of her face, trunk and

extremities, including grimace-like facial tics and frequent

tonic upward eye deviation. These movements were not distressful

to her. She endorsed a "good" or "fine" mood throughout her

stay. We were in contact with her outpatient movement disorders

specialist [REDACTED] MD) who recommended continuing

antipsychotic therapy as we were doing. We ultimately settled on

a Risperidone dose of 0.5mg BID, slightly lower than her

original dose of 0.75mg BID with which she had been

noncompliant. She has additional 0.25-5mg PRN BID as needed for

agitation, but actually throughout hospitalization she was^[C]

pleasant and cooperative and did not show any irritability,^[C]

impulsivity or aggression^[C]. She is being discharged to an

assisted living facility that specializes in patients with

[REDACTED] Disease, as had been previously arranged by her

family prior to this. Will follow up on [REDACTED] with Dr.

[REDACTED].

LEGAL STATUS

The pt remained on a CV

PSYCHO**SOCIAL^[C]**

The team coordinated care w/ the pt's family who were supp^[C]ortive

and arranged ongoing housing/care at a [REDACTED] facility

upon the pt's d/c.

The pt was visited on the unit frequently by her friend [REDACTED],

whom she found to be supportive.

Ms. [REDACTED] was visible on the unit during her stay here, and

although she had **difficulty w/ verbalizing thoughts/feelin^[A]gs** [REDACTED]

her **[REDACTED], she was seen to be interacting we^[A]ll w/ others**

in the dayroom. She never behaved in an unsafe manner and

participated in the unit. She also was groomed and dressed in casual clothing on the day of her discharge.

RISK ASSESSMENT

Ms. [REDACTED] has a low risk of imminent harm/aggression/violence.

She **has been calm and in very good behavioral control throughout the entirety of her psychiatric admission. She consistently denied** any si/hi and showed an ability to care for herself safely despite the intensity of an acute inpatient psychiatric environment.

She does, however, have a high risk of future decompensation given the fact that she has a progressive neurodegenerative condition that is associated w/ mood swings and psychosis in its later stages.

We helped to mitigate that risk by resuming Ms. [REDACTED] antipsychotic medication and would recommend that she continue to take it going forward. We also added some prn Risperdal in the event that she has experiences future episodes of aggression/instability. We also reduced her risk **of harm to self/others, by d/c'ing Ms. [REDACTED] to a facility where she will have** access to professional staff and monitoring. She also has f/u arrangements made with providers to continue to manage her [REDACTED].

Prognosis:

Guarded

Medications on Admission:

The Preadmission Medication list is accurate and complete.

1. RISperidone 0.75 mg PO BID

Discharge Medications:

1. RISperidone 0.5 mg PO BID

RX *risperidone 0.5 mg 1 tablet(s) by mouth twice a day Disp

#*60 Tablet Refills:*0

2. RISperidone 0.25-0.5 mg PO BID:PRN agitation

RX *risperidone 0.25 mg [REDACTED] tablet(s) by mouth twice a day Disp

#*60 Tablet Refills:*0

Discharge Disposition:

Extended Care

Facility:

[REDACTED]

Discharge Diagnosis:

- Mood disorder secondary to General Medical Condition
- Advanced [REDACTED] Disease

Discharge Condition:

Mental Status: Confused - sometimes.

Level of Consciousness: Alert and interactive.

Activity Status: Ambulatory - Independent.

Discharge Instructions:

Dear [REDACTED],

You were admitted to the inpatient psychiatric unit at [REDACTED] for worsening agitation and difficulty caring for yourself at home, due to your [REDACTED] Disease. We restarted your Risperidone which was very helpful and improved your behavior significantly. You will be discharged to an assisted living facility in [REDACTED] that specializes in patients with similar health problems.

We made the following changes to your medications:

- RESTARTED your Risperidone at a slightly lower dose: 0.5mg twice daily, plus an additional 0.25-0.5mg as needed for agitation.

Followup Instructions:

[REDACTED]

Detailed Clinical Assessment

1. Patient Overview

The patient is a female with a primary diagnosis of Mood Disorder secondary to General Medical Condition, specifically Chorea. She was admitted due to increased agitation, impulsivity, and difficulty caring for herself. Key features include chronic stable progression of her mood disorder, significant weight loss, and a history of aggression and refusal to take medication.

2. Mental Health Status

The patient's current mental health state is characterized by moderate suicidal ideation, aggression, and self-injury denial. Her mood disorder is secondary to her medical condition, which complicates her psychiatric care.

3. Key Risk Factors

- **[MOD]** Chronic stable progression of mood disorder
- **[HIGH]** Moderate suicidal ideation with active threats
- **[LOW]** Aggression and refusal to take medication
- **[LOW]** Imminent homelessness and housing loss
- **[LOW]** Significant weight loss and functional decline

4. Protective Factors

- ✓ Engagement with a multidisciplinary team including psychiatrists and social workers
- ✓ Medication with Risperdal for Chorea movements
- ✓ Reduced risk of harm to self/others by discontinuing Ms. [REDACTED]'s medication

5. Counterfactual Scenario Briefing

Each card represents a potential deterioration pathway. Content is preserved exactly as generated — only formatting is enhanced.

■ [A] Psychiatric / Clinical Deterioration Pathway

Plausibility: HIGH

The patient's current clinical state, including her chronic mood disorder secondary to a general medical condition, coupled with her recent functional decline and housing crisis, presents a high plausibility for negative trajectory. Her history of aggression, refusal to take medication, and significant weight loss are indicative of an imminent risk of further deterioration. MH RELEVANCE: high

Warning Signs:

- Increased confusion and agitation
- Refusal to adhere to medication regimen
- Further significant weight loss or malnutrition
- Worsening ataxic gait or falls
- Expressions of hopelessness or severe depression

Crisis Endpoint: The patient could experience a severe decompensation event, potentially leading to acute psychiatric crisis, self-neglect, or harm to others due to her deteriorating physical and mental health status.

Preparedness Actions:

1. Implement a structured daily routine with assistance to ensure medication adherence
2. Arrange for a home health aide or increased nursing care to monitor her physical health
3. Engage a multidisciplinary team, including a psychiatrist, neurologist, and social worker, to address her complex medical and psychiatric needs
4. Explore options for stable housing to reduce homelessness-related stress
5. Develop a crisis intervention plan, including emergency contact information and steps for involuntary hospitalization if necessary

The patient, currently diagnosed with a mood disorder secondary to a general medical condition, is at risk of experiencing exacerbated symptoms due to the progression of her underlying [REDACTED] Disease. Given her recent significant weight loss, ataxic gait, and monosyllabic speech, there is a con...

■ [B] Substance Use Escalation Pathway

STATUS: NOT APPLICABLE

NOT APPLICABLE — No substance use evidence. Data: alcohol=none, drugs=none, tox_screen=negative. All toxicology screens negative. Generating a substance abuse scenario would be clinically unfounded.

■ [C] Social / Environmental Collapse Pathway

Plausibility: **HIGH**

Warning Signs:

- Increased aggression or impulsivity
- Withdrawal from social interactions
- Significant changes in sleep patterns (insomnia or hypersomnia)
- Expressions of hopelessness or severe depressive thoughts
- Deterioration in self-care and daily functioning

Crisis Endpoint: The patient may experience a severe psychotic break, characterized by intense delusions or hallucinations, due to the extreme stress of homelessness and trauma, coupled with her pre-existing mood disorder. This could lead to a dangerous situation where the patient is unable to care for herself or may harm others, necessitating immediate intervention.

Preparedness Actions:

1. Arrange for a temporary shelter or transitional housing to provide immediate stability
2. Coordinate with a multidisciplinary team, including mental health professionals, to develop a comprehensive care plan
3. Initiate or adjust psychotropic medication to manage mood symptoms and potential psychotic features
4. Provide trauma-informed care and support, considering the patient's trauma history
5. Engage with community resources, such as social services and support groups, to build a network of support for the patient

The patient, currently homeless and with a significant weight loss, is at risk of experiencing severe mental health deterioration due to the imminent loss of her only stable housing. The stress of homelessness, combined with her existing mood disorder secondary to a medical condition, could lead to ...

6. Priority Preparedness Actions

Immediate clinical interventions recommended based on risk assessment.

1. Arrange for a temporary shelter or transitional housing to provide immediate stability.
2. Implement a structured daily routine with assistance to ensure medication adherence.
3. Engage a multidisciplinary team, including a psychiatrist, neurologist, and social worker, to address her complex medical and psychiatric needs.
4. Coordinate with a home health aide or increased nursing care to monitor her physical health.
5. Adjust psychotropic medication to manage mood symptoms and potential psychotic features.

6. RISK TIER: HIGH

7. Risk Assessment Summary

OVERALL RISK TIER: **HIGH**

The patient's risk is rated as high due to her chronic stable mood disorder secondary to a medical condition, active suicidal ideation, aggression, and imminent homelessness, all of which significantly impact her mental health and risk of harm to self or others. [OVERRIDE: Risk tier elevated to HIGH due to critical safety signals detected in clinical note: CRITICAL SIGNALS DETECTED: suicide (1 signal), violence (3 signals), self_harm (1 signal). Risk override: HIGH.]